Moneymeds

Moneymeds: Executive Summary

Problem

Many young adults with chronic conditions have to make difficult choices about their medication. Aging off a parent's insurance plan, working part-time or gig jobs, or enrolling in a high-deductible plan often leaves them paying a large share of prescription costs themselves. Surveys of patients regularly report that a meaningful portion of people skip doses, split pills or delay refills because of cost. For someone managing asthma, diabetes or a mental health condition, these choices can lead to emergency visits that cost far more than the medication would have.

The tools that could help are fragmented. The cash price of one prescription can differ a great deal from one pharmacy to the next, even within a single neighborhood. Discount cards, manufacturer savings coupons, patient assistance programs and pharmacy loyalty programs each have their own rules, and no single place tells a patient which one to use. Young adults in particular are often new to managing their own health costs and have had little guidance on how to do it.

Solution

Moneymeds is a mobile app that does three things in one place. First, it compares prescription prices across nearby pharmacies based on the user's medications and location. Second, it identifies which discount program or coupon applies and shows the resulting price, so the user does not have to test several cards at the counter. Third, it offers a simple monthly health budget that accounts for refills, copays, deductibles and upcoming appointments, and sends reminders before a refill is due so users can shop around ahead of time.

The app is designed for people who are not experts in healthcare. We use plain language, short onboarding and a focus on the few decisions that matter most: where to fill a prescription, which program to use and how much to set aside each month. We also include short educational content explaining terms such as deductible, formulary and generic substitution, since many of our target users have never had to learn them.

Market

Our target customers are U.S. adults ages 18 to 34 with at least one chronic condition requiring ongoing prescription medication. Public health data suggests that a substantial share of young adults live with a chronic condition, and a large portion of them report some difficulty affording care. We estimate that several million people fall in this group nationally, though we have not yet refined this figure. Our initial focus is students and recent graduates in the region around our university, where we can reach users through campus health centers, student groups and local clinics.

Beyond individual consumers, we see secondary markets in campus health services, employers who want to support younger employees, and community clinics that help patients find affordable medication. We have not yet tested interest in any of these.

Business Model

We plan to earn revenue in two ways. The first is referral fees paid by discount program providers and pharmacies when a user fills a prescription through a Moneymeds recommendation. The second is a premium subscription of a few dollars per month offering features such as family sharing, expanded budgeting tools and refill planning across multiple medications. The core price comparison would remain free so that cost is never a barrier for the users we most want to reach.

We recognize that referral revenue depends on agreements we have not yet signed, and that

conversion from free to paid is uncertain. Our current financial thinking assumes modest early revenue from referrals and a small share of users converting to the premium tier, and we intend to refine these assumptions with pilot data. Our main costs will be software development, data licensing for pharmacy prices and basic cloud infrastructure.

Competition

The prescription savings space is crowded. Several well-known discount card and coupon services already let people look up prices and print or show a card at the pharmacy. Large pharmacy chains and insurers have their own apps, and some online pharmacies offer low-cost generics by mail. Most of these services are designed for the general population and earn money by taking a share of each transaction.

We believe Moneymeds can differentiate itself in three ways. First, we focus on young adults with chronic conditions rather than all consumers, which lets us tailor the experience and the education to their situation. Second, we combine price comparison with budgeting, whereas most competitors stop at the pharmacy counter. Third, we compare across several discount sources rather than promoting a single one. We acknowledge that larger competitors could add similar features, and that our advantage will depend on how well we understand and serve this specific group.

Team

Priya Raman, chief executive officer, is a senior studying public health. She has volunteered at a campus health center and leads product direction and user research. Daniel Okafor, chief technology officer, is a computer science senior who built the first version of the app and manages development and data integration. Maya Lindqvist, chief operating officer, is a junior in business administration who handles partnerships, outreach and early financial planning. We have also had informal conversations with a faculty member in health policy and a local pharmacist, who have offered feedback but are not formally involved.

Milestones and Financials

We have defined our core concept and formed a Delaware C corporation. We have built a working prototype that compares prices for a limited set of common medications using publicly available and sample data. We launched the prototype with our first outside users, a small group of students recruited through campus contacts, and have begun collecting feedback on how they use it. Early comments suggest users value seeing price differences and want better budgeting reminders, though our sample is small and we have not yet measured savings or retention. We have a basic website describing the product.

To date we have spent only a few thousand dollars, funded by the founders, mainly on legal formation, hosting and data access. We have no revenue. Over the next twelve months we expect to need roughly seventy-five thousand dollars to expand data coverage, improve the app, run a larger pilot and begin discussions with discount providers about referral agreements. We have not yet built a detailed financial model beyond these estimates.

What We Are Asking For

We are applying to this competition for funding, mentorship and connections. Prize money would go toward licensing more complete pharmacy price data and running a pilot with several hundred users. We would especially value guidance from mentors with experience in healthcare regulation, privacy and partnership agreements, since we are still learning how to handle sensitive health information responsibly. We would also welcome introductions to campus health centers, clinics and potential discount program partners. Our goal for the coming year is to show that users save real money and keep using the app, which would give us a stronger basis for raising outside investment.